My plan until I see the doctor

Rimon · Wed 21 Oct to Fri 30 Oct 2026 · EEE-221 retake on Thu 29 Oct

The 4 rules for this week

1. **Small every day beats big once.** A good day is not a day to do more. Doing too much is what triggers the 3 to 7 day crash.
2. **Medicines exactly as prescribed.** Do not stop, skip or change any dose yourself. Questions go to Prof. Huq.
3. **EEE-221 comes first.** Failing a special retake costs your seat. No 3rd-semester study this week.
4. **A floor day still counts.** On a crashed day, do only the "minimum day" below and call it a success.

1. The jobs for this week

☐ **Call Prof. Huq's chamber for a follow-up this week** (use the serial number at the bottom of your 5 Sep prescription). Your 1-month check was missed and you have been on two SSRIs for about 6 weeks. If he cannot see you before the exam, ask for a phone call. The Dhaka question can wait; this cannot.

☐ **Prepare a doctor folder:** all prescriptions, the uroflowmetry report, the ultrasound report if you can find it, your diagnosis report, and this week's log (last page).

☐ **Send the department message** from your exam plan (DLD lab make-up, medical route for retakes, missed CTs, past papers). One message, once.

☐ **Ask your family two things:** do I snore or stop breathing in my sleep? Do my legs jerk or move a lot at night?

☐ **Find your safe walk** (section 4) and log it every day.

☐ **If you own or can borrow a BP machine,** do the stand test once (section 6) and bring the numbers.

Go to emergency the same day (call 999) if

- You faint and hurt yourself, or faint without warning or while lying down.
- A lot of rectal bleeding, or black, tarry stools.
- Heavy sweating together with fever, racing heart, confusion, or shaking or twitching that spreads or will not stop.
- Sudden loss of vision, many new floaters or flashes, or a curtain over your vision.
- Thoughts of ending your life: tell someone at home the same day, and call 999 if you feel unsafe.

2. Shape of a day

When	What
Wake (same time every day, within 30 min, weekends too)	Pick one time and keep it, e.g. 7:30. Do not sleep in on Friday and Saturday; that is what scrambles your sleep clock.

Morning	Medicines as prescribed. Breakfast with an egg. **10 to 15 minutes outside in daylight** before 10 am (sitting on the roof or balcony is fine). This sets your sleep clock and helps vitamin D.
Bathroom	Follow section 5. Afterwards, **rest 15 minutes lying down** before anything else.
Study blocks	EEE-221 only, per your exam plan. Each block 25 minutes, phone in another room, 10+ minutes lying down between blocks. Use Zuhr and Asr as natural break points.
Walk	Your safe walk (section 4), in the day, not straight after a long bathroom session.
Evening	Write one line in the log (last page). Lighter dinner than lunch. Less water after 9 pm.
Bed	Aim for 12:00 this week, 11:30 next week. Phone charging outside the bed. Lie on your side.

Day types

- **Floor day** (crashed, foggy): 1 study block of 15 min, no walk, sunlight if you can, early night.
- **Normal day:** 3 study blocks, safe walk, done by early evening.
- **Good day:** maximum 5 blocks. Stop even if you feel great.

Minimum day (always enough)

- Medicines as prescribed
- Daylight for 10 minutes
- 1 study block
- One line in the log

3. Day by day

Day	Health	Study (EEE-221)
Wed 21 Oct	Call Prof. Huq's chamber. First safe-walk test: 5 minutes, easy pace.	Day type decides blocks.
Thu 22 Oct	Note how you feel after yesterday's walk. Send department message. Ask family the 2 questions.	Blocks per day type.
Fri 23 Oct	Same wake time as weekdays. Jumu'ah. Prepare the doctor folder.	Light day: 2 blocks max.
Sat 24 Oct	Same wake time. Stand test if you have a BP machine.	Blocks per day type.
Sun 25 to Tue 27 Oct	Safe walk daily (same length all week). Doctor visit whenever booked.	Past questions, written by hand.
Wed 28 Oct (exam eve)	Bag ready at the door: admit card, ID, pens, calculator, water, snack, cash for a CNG. Work out wake time from your **worst** recent bathroom time plus commute. Bed by 11:30.	2 light review blocks. Nothing new.
Thu 29 Oct	Eat before leaving. Go easy on tea.	**EEE-221 exam.** Answer what you know first; stuck 5 min, move on.

Fri 30 Oct	Rest day. Short safe walk only if you feel fine.	No study.

4. Walking ladder (pacing, not pushing)

1. **Start at 5 minutes** on flat ground, at a pace where you can talk easily without getting out of breath.
2. **Next day, check:** if you feel no worse than usual, 5 minutes is your safe walk. If you crashed, drop to 3 minutes, or 2 minutes twice.
3. **Keep the same length every day this week,** even on good days. Do not add during exam week.
4. After the exam, add 2 to 3 minutes only after a full week with no crash. Two short walks are easier than one long one.
5. **Rickshaw for the long part is allowed.** It is part of the plan, not laziness.

What you can say at home: "My doctors are checking whether my medicines are causing the fatigue. Meanwhile I walk a small amount every day and increase it a little each week, because pushing hard makes me crash for days."

5. Bathroom

- **Urinate standing,** since that helped you most. **Go every 3 to 4 hours while awake**, even without an urge. Going 12 hours lets the bladder over-stretch and lose its signal.
- **For stool, raise your feet on a low stool or bucket** (knees above hips). This straightens the rectum and needs less pushing.
- **Breathe out slowly and let your belly go soft**, as if sighing. Do not hold your breath and strain hard: that is what drops your blood pressure and makes you faint.
- **The feeling of "not finished" is not reliable.** Your ultrasound showed the bladder empties. When the main part is done, try to leave even if the feeling is still there. Write down the time spent; aim to shave just a few minutes off your usual, not to be perfect.
- **Getting up after:** sit for 1 minute first, stand slowly with a hand on the wall. If your vision greys out, sit or lie down immediately. Do not lock the door.
- Never walk or study straight after a long session: rest 15 minutes lying down.

6. Sleep, food and body

Sleep

- Same wake time every day (most important).
- Phone out of the bed. No anime after 11 pm.
- If your nose is blocked at night, tell the doctor; mouth-breathing may explain the drooling and morning headaches.
- If daytime sleepiness hits, a 20-minute nap before 3 pm is fine. Longer naps break the night.

Food swaps

- **1 egg every day** (boiled is fine).
- **A fruit or a handful of nuts instead of Oreos.**
- Milk or doi once a day if you can.
- Keep fish, dal and shak: they are good.
- Smaller rice portion at lunch on study days to reduce the afternoon slump.
- Water as now (2 to 2.5 litres), mostly before 9 pm.

Restless legs at night

- Note in the log whether it happened.
- A short leg stretch or warm water on the legs before bed may help.
- Tell Prof. Huq, together with the leg shaking when your foot rests on a bar.

Stand test (only if you have a BP machine)

1. Lie down 5 minutes, then measure pulse and BP.
2. Stand up, keep still, measure at 1, 3, 5 and 10 minutes.
3. Write all the numbers down. Sit at once if you feel faint.

A rise of 30+ beats per minute is worth showing a medicine doctor.

7. For the doctor visit

Say this first: "I was not tired all day before 2023. The crashes and daytime sleepiness started after my psychiatric medicines began, and got worse when the dose went up in 2024."

Questions for Prof. Huq

1. I have had rectal bleeding several times since starting Prodep, and three nosebleeds since 2023. Is my bleeding risk higher on two SSRIs?
2. On the first Prodep day I had severe cramps and anal spasms. I sweat heavily at rest. My leg shakes rhythmically when my foot rests on a bar, and I get jerks when falling asleep. Could these be too much serotonin? Can you check my reflexes?
3. I shake my legs at night to settle and feel restless. Is this restless legs or akathisia?
4. I fall asleep sitting in class and exams after 8 hours of sleep. Could my medicines or their timing cause this? What can be done before my exam on 29 Oct?
5. Washing improved but bathroom time did not. Can I get hands-on ERP for the bathroom with a trained therapist? If Dhaka, please name the person or centre.
6. I occasionally take montelukast (Provair) for cough. Is that okay with my condition?
7. Could you write a short note about the Prodep reaction on the day of my DLD lab final?

Tests to ask for (from him or a medicine doctor)

Ferritin, vitamin D (25-OH), vitamin B12, TSH and free T4, serum sodium and potassium, creatinine, ALT, HbA1c, CRP, and a repeat CBC if your last one was before September. Also ask whether a rectal examination is needed for the bleeding, and whether the Eid 2024 episode needs an EEG.

8. Daily log (one line each evening)

Day type: F = floor, N = normal, G = good. Energy is how you feel the morning after, 1 (worst) to 10 (best). Send me a photo or type the line in the thread.

Date	Day type	Slept / woke	Study blocks	Walk (min)	Sunlight (Y/N)	Longest bathroom (min)	Restless legs (Y/N)	Energy next morning	Note
Wed 21									
Thu 22									
Fri 23									
Sat 24									
Sun 25									
Mon 26									
Tue 27									
Wed 28									
Thu 29									
Fri 30									

Bathroom times (write each visit if you can)

Date	Urination times (clock time, minutes)	Stool (clock time, minutes, hard / soft)
Wed 21		
Thu 22		
Fri 23		
Sat 24		
Sun 25		
Mon 26		
Tue 27		

This plan is guidance to carry you until your doctor visit, not a diagnosis or medical advice. Medicines are changed only by your doctor. May Allah make it easy for you.